AWARENESS OF REGISTERED MEDICAL PRACTITIONERS ON CERTIFICATION OF SEXUAL POTENCY – A QUESTIONNAIRE-BASED STUDY

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Introduction: The question of potency or impotency arises in both civil and criminal cases.There is an increasing trend in taking of impotence/potency as a defense in various legal issues.Therefore, this study was done to evaluate the level of awareness on certification of impotency/potency among registered medical practitioners

Methodology: This study was a prospective, cross-sectional, questionnaire-based study.After obtaining consent from the participants, a questionnaire was administered which had 10 questions to assess the awareness in addition to questions related to demographics and previous experience. The proportions of participants with awareness and without awareness for each domain is presented.

Results: The total number of registered medical practitioners enrolled into study were 146. The maximum number of participants, 53, were in age group of 35-45 years. As high as 87% of participants had awareness regarding laboratory tests conducted for potency. However, only 12% had awareness regarding methods for measuring potency.

Conclusion: While basic knowledge of laboratory assessments is relatively high, awareness of advanced diagnostic tools and procedural standards is limited.

key words: Potency, Impotency, Medicolegal Certification, Medical Practitioner Awareness

INTRODUCTION

The definition of sexual potency is the ability to carryout and consummate sexual intercourse, usually referring to the male. The opposite of sexual potency is impotency/ erectile dysfunction, which is defined as the inability of a person to perform sexual intercourse. The question of potency or impotency arises in both civil and criminal cases.[1,2]

Nullity of marriage, divorce and disputed paternity are the civil cases where a plea of potency is taken, whereas in criminal cases such as sexual offences, the medical examination of the accused includes general examination, potency and collection of evidence, where a plea of impotency is taken.[2,3]

There is an increasing trend in taking of impotence/potency as a defense in various legal issues. The actual data regarding the number of civil and criminal cases in which the plea of potency/impotency is taken is not available.

In general the civil cases are generally referred to the head of the hospital and criminal cases to the head of the forensic medicine & toxicology. But according to literature there is no evidence that civil should be seen by a certain specialty and criminal cases should be seen by some other specialty. As per the standard literature, the examination for potency in medicolegal cases should be undertaken only when asked by the court or police. [2]

The examination of a case of potency/ impotency is included in the undergraduate curriculum, but still the level of awareness is very low and in turn leading to numerous complications. [2] This is the reason why we have tried to assess the level of awareness among registered medical practitioners towards certification of potency/ impotency.

METHODOLOGY

This study was a prospective, cross-sectional, questionnaire-based study done at department of forensic medicine & toxicology of a medical college at Telangana. This study was initiated after obtaining Institutional Ethics Committee approval and written informed consent from participants.

The study objectives were to assess the level of awareness on certification of impotency/potency among registered medical practitioners and to assess the awareness of registered medical practitioners about rigiscan. Registered medical practitioners willing to participate in the study were included.

After obtaining consent from the participants, a pre-tested semi-structured questionnaire was administered which had 10 questions to assess the awareness on certification of impotency/potency in addition to questions related to demographics and previous experience of certification. Data was collected through anonymised printed questionnaires. Data was accessible only to study team.

Demographic characteristics like age, sex, qualification, previous experience in certification, are summarized using frequency distribution and percentages. The proportions of participants with awareness and without awareness for each domain has been presented.

RESULTS

The total number of registered medical practitioners enrolled into study were 146. The maximum number of participants, 53, were in age group of 35-45 years and minimum number of 7 participants belonged to 65-75 years. Of the 146 participants, 59 were female and 87 were male. 11 participants had previous experience. Majority of participants, 120 were having higher qualification as MD and 108 were working in Government. The details are presented in Table No 1

Parameter	Participants (N=146)
Age groups	**n(%)**
25-35	14 (9.6)
35-45	53 (36.2)
45-55	48(32.9)
55-65	24(16.5)
65-75	7(4.8)
Sex	**n(%)**
Female	59 (40.4)
Male	87 (59.6)
Qualification	**n(%)**
MBBS	8 (5.5)
MD/MS	120 (82.2)
DM/MCh	18(12.3)
Institution	**n(%)**
Government	108(73.9)
Private	38(26.1)
Previous Experience of Issuing potency certificates	**n(%)**
Present	11(7.5)

Table No 1: showing parameters of participants

All the enrolled 146 participants answered the questionnaire. It was observed there was high level of awareness (87%) regarding laboratory tests conducted for potency, such as hormonal assays and semen analysis. However, only 23% of respondents believed that tests such as RigiScan or papaverine injection are mandatory for certifying potency and 84% of practitioners felt that positive test results are sufficient to declare an individual potent.In addition, only 12% had awareness regarding methods for measuring potency. Furthermore, less than half of the participants (47%) were aware of the proper format for certification of potency or impotency. The details of the responses to eight different questions on awareness is presented in Fig No 1

Of the participants who were aware about the laboratory test conducted for potency, 76% were aware about hormonal analysis, 18% were aware of semen analysis and 6% were aware of both the

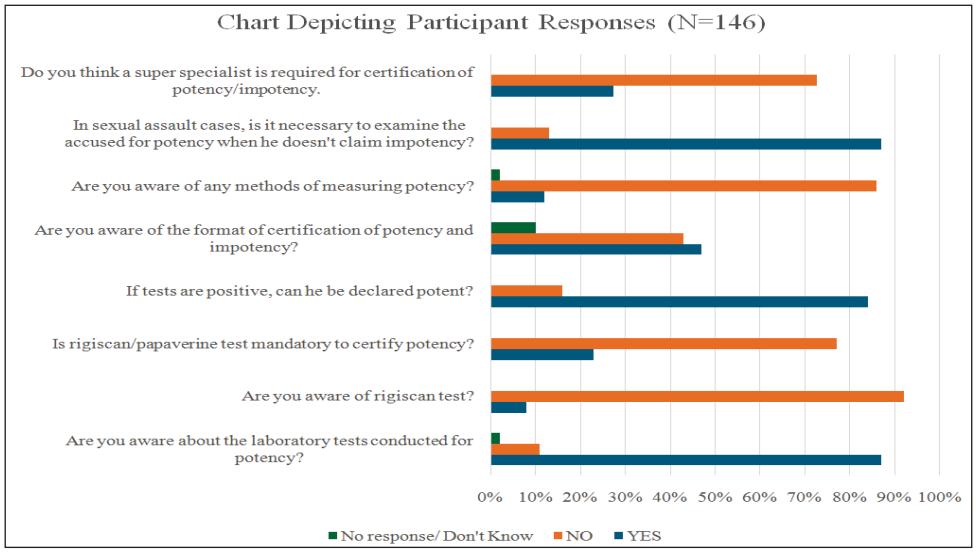

Figure No 1 depicting participant responses

tests. Of the participants who felt a superspecialist is required for certification of potency/impotency, 58% thought an endocrinologist is required while 42% thought urologist is required.

DISCUSSION

The findings of this study reveal important insights into the awareness levels of registered medical practitioners concerning the certification of potency and impotency. Despite the inclusion of this topic in undergraduate curricula, substantial gaps persist, particularly concerning modern diagnostic techniques and procedural protocols.

A notable finding was the high level of awareness (87%) regarding laboratory tests conducted for potency, such as hormonal assays and semen analysis. This suggests that basic knowledge of standard laboratory evaluations is widely disseminated. However, the awareness regarding advanced diagnostic tools like the Rigi Scan test was strikingly low, with only 8% of practitioners indicating familiarity. The RigiScan, a specialized test for nocturnal penile tumescence, is often cited as a valuable tool for diagnosing erectile dysfunction. Rigiscan is the best available and objective test to differentiate between ED of organic and psychogenic etiologies.[4] This gap underscores the need for focused training on advanced diagnostic methodologies and their utility in clinical and forensic settings.

Interestingly, only 23% of respondents believed that tests such as RigiScan or papaverine injection are mandatory for certifying potency. This aligns with the broader understanding that while these tests may provide valuable insights, their routine application for certification is not universally mandated. The finding that 84% of practitioners felt that positive test results are sufficient to declare an individual potent highlights a potential over-reliance on test outcomes without considering holistic evaluations. In a study done by Shilpa D etal., on evaluation of rationale of conducting penile color doppler with papaverine injection in medicolegal cases of potency, it was opined that every accused of sexual assault should not be subjected to penile color doppler study and only the accused asserting impotency should undergo potency test and color doppler ultrasound. It was also suggested that screening methods for impotency like visual erotic stimulation, NPTR (nocturnal penile tumescence and rigidity) should be done. Penile color doppler with papaverine injection should be done in cases found impotent on screening.[1]

It is important to emphasize that a person may appear physically normal and still be presumed potent unless proven otherwise. Therefore, in certain medicolegal scenarios—particularly when

no overt complaint of impotence exists—it may not be necessary to conduct an elaborate diagnostic evaluation. If the individual in question demonstrates normal secondary sexual characte ristics, is not suffering from any gross physical debility, and has no visible local diseases or congenital/acquired malformations of the genitalia, it is medically reasonable to certify that there is nothing to suggest that he is incapable of performing sexual intercourse. Routine certification does not require the employment of specialized tools such as the RigiScan or penile Doppler unless specifically indicated especially when impotence is claimed as a defense, or the court explicitly mandates advanced testing. A thorough physical and local examination, combined with a proper sexual and medical history, is often sufficient.

Such an approach has support in both medico-legal literature and clinical practice, wherein over-reliance on advanced tests may not only be unnecessary but may lead to undue delays and ethical concerns. Therefore, in the absence of clear medical or legal indications requiring advanced testing, certification based on physical examination remains an accepted and practical standard. [5]

The results also revealed low awareness levels (12%) regarding methods for measuring potency. This deficiency in procedural knowledge may contribute to inconsistencies in the application of certification standards across institutions. Furthermore, less than half of the participants (47%) were aware of the proper format for certification of potency or impotency, reflecting a need for standardized documentation training.

In cases of sexual assault, 87% of practitioners affirmed the necessity to examine the accused for potency even if no claim of impotence was made. This aligns with medico-legal guidelines emphasizing comprehensive evaluation to establish credibility in such cases. [2] However, the lack of consensus on mandatory tests and procedural adherence suggests the need for unified protocols and better enforcement of guidelines.

In this study, of the 27% participants who felt super specialist is needed, 58% thought an endocrinologist is required while 42% thought urologist is required. Ideally for criminal cases referred for potency test, there should be a medical board to assess the person. The board should comprise urologist, psychiatrist, general surgeon, radiologist, physician, and forensic medicine experts for the complete assessment as the causes of impotence could be organic or psychological. However, a crucial point to consider is that in India, the majority of medical examinations are carried out in government-run primary health centres, community health centres, and district hospitals by doctors who typically hold only an MBBS degree, the basic medical qualification.[3]

CONCLUSION

This study highlights significant gaps in the awareness and application of protocols for certifying potency and impotency among registered medical practitioners. While basic knowledge of laboratory assessments is relatively high, awareness of advanced diagnostic tools and procedural standards is limited. These findings call for urgent measures to address these deficiencies through targeted education, standardized training programs, and curriculum reforms.

Key recommendations include:

1. Curriculum Enhancement: Integrating advanced diagnostic tools, such as RigiScan, into the medical education curriculum.
2. Standardized Guidelines: Developing and disseminating clear, standardized protocols for the certification of potency/impotency, including documentation formats.
3. Workshops and Training: Conducting regular workshops and continuous medical education programs focusing on forensic medical examinations, particularly in potency certification.
4. Specialist Collaboration: Encouraging interdisciplinary collaboration with specialists, such as endocrinologists and urologists, for complex cases.

5. Policy Enforcement: Strengthening adherence to medico-legal examination protocols across institutions to ensure consistency and credibility.

By addressing these gaps, regulatory authorities can enhance the preparedness of medical practitioners, thereby improving the reliability and validity of potency certifications in both civil and criminal cases.

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